CaseNote Lab

OET Writing workbook for nurses

Free sample case · full pack (24 cases + app) at kitwisestudio.github.io/casenote-lab

How to use this workbook: read the case notes for 5 minutes without writing, then write your letter in 40 minutes on the lined page (body about 180–200 words). Afterwards, type it into the CaseNote Lab app to check which key notes you used, then compare with the model letter at the back.

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T05	Stroke – transfer to rehabilitation unit	Transfer	medium

T05 · Stroke – transfer to rehabilitation unit

Acute stroke unit · Transfer · medium · Today's date: 5 March 2026

Writing task. Using the information in the case notes, write a letter to Ms Niamh Doyle, Clinical Nurse Manager at the Rehabilitation Unit, Ardmore Rehabilitation Centre, where Mr Quinlan is being transferred today for ongoing rehabilitation. Address the letter to Ms Niamh Doyle, Clinical Nurse Manager, Rehabilitation Unit, Ardmore Rehabilitation Centre, Ardmore Road, Limerick. In your answer: expand the relevant notes into complete sentences; do not use note form; use letter format. The body of the letter should be approximately 180–200 words.

SOCIAL BACKGROUND

- Married – wife Mary visits daily, wants to be involved in care
- Dairy farmer – son now running farm
- 2-storey farmhouse, bathroom upstairs
- Enjoys watching hurling
- Private health insurance

MEDICAL HISTORY

- Hypertension, hypercholesterolaemia
- Inguinal hernia repair 2015
- Wears reading glasses
- NKDA

ADMISSION

- 18/02/2026 – L MCA ischaemic stroke; thrombolysed (alteplase)
- 24-hr CT scan – no haemorrhagic transformation
- Aspirin 300mg x 14 days – completed 04/03

CURRENT CONDITION

- R-sided weakness: R arm 2/5, R leg 3/5
- Expressive dysphasia – understands simple commands; uses yes/no & picture board
- Dysphagia – SALT: IDDSI level 5 minced & moist diet, level 2 mildly thick fluids
- Meds: clopidogrel 75mg od, atorvastatin 80mg nocte, ramipril 2.5mg od
- Transfers: assist x2 with standing aid; sits unsupported
- High falls risk – bed sensor alarm in use
- Urinary catheter removed 25/02 – incontinent at night, pads
- Bowels: lactulose 10ml bd, BO alternate days
- Waterlow 18 (high risk) – 2-hrly repositioning; sacrum intact
- OT: needs assist x1 with upper body washing/dressing
- Tearful at times – post-stroke mood screen pending
- Wt 78kg, stable

TRANSFER PLAN

- Goals: improve mobility & communication; return home
- Transfer by ambulance 05/03/2026, 10:00

Your letter

Your letter

Model letters

Try each task yourself before reading its model letter.

T05 · Stroke – transfer to rehabilitation unit

Ms Niamh Doyle
Clinical Nurse Manager
Rehabilitation Unit
Ardmore Rehabilitation Centre
Ardmore Road
Limerick

5 March 2026

Dear Ms Doyle,

Re: Mr Patrick Quinlan, DOB 12/04/1958

I am writing to transfer Mr Quinlan, who had a left middle cerebral artery ischaemic stroke on 18 February, to your unit for ongoing rehabilitation. His goals are to improve his mobility and communication and to return home.

Mr Quinlan has right-sided weakness (arm 2/5, leg 3/5) and requires the assistance of two staff and a standing aid for transfers. He is at high risk of falls, so a sensor alarm is in use. He also has expressive dysphasia; however, he understands simple commands and communicates using yes/no answers and a picture board.

Following speech and language therapy assessment, he requires a minced and moist diet (IDDSI level 5) and mildly thick fluids (level 2). His medications include clopidogrel 75mg, atorvastatin 80mg and ramipril 2.5mg daily.

His urinary catheter was removed on 25 February, and he is now incontinent at night, for which pads are used. His Waterlow score is 18, so he requires two-hourly repositioning; his sacrum is currently intact.

He has been tearful at times, and a mood screen is pending. His wife visits daily and wishes to be involved in his care.

Please contact me if you require any further information.

Yours sincerely,

Staff Nurse
Acute Stroke Unit
Shannonside University Hospital

WHY IT IS WRITTEN THIS WAY

- The opening gives the diagnosis, the reason for transfer and the patient's rehabilitation goals, so the reader immediately understands the aim of care.
- Information is grouped by care need: mobility and falls, communication, swallowing and medication, continence and skin, then mood and family.
- The normal follow-up CT scan is a tricky distractor – it reassures but does not change rehabilitation nursing.

- Farming background, hobbies, insurance, reading glasses and old hernia repair are not needed by the rehabilitation team.
- Specific details (IDDSI levels, Waterlow score, two-staff transfers) are kept exact because the receiving nurses must act on them safely.
- Useful phrases: 'requires the assistance of two staff', 'for which pads are used', 'wishes to be involved in his care'.